

Poikiloderma and Parapsoriasis Variants

Lesions may appear finely wrinkled—referred to as "cigarette paper" wrinkling—exhibiting varying degrees of epidermal atrophy. When atrophy becomes prominent, telangiectasia and mottled pigmentation are also observed. This triad of **atrophy**, **mottled pigmentation**, and **telangiectasia** defines the term *poikiloderma* or *poikiloderma atrophicans vasculare*, which can also be seen in other conditions.

Retiform Parapsoriasis

Retiform parapsoriasis is a rare variant of large-plaque parapsoriasis (LPP) characterized by an extensive eruption of scaly macules and papules arranged in a net-like or zebra-stripe pattern. Over time, these lesions evolve into a poikilodermatous appearance.

Small-Plaque Parapsoriasis (SPP)

SPP typically presents as round or oval, discrete patches or very thin plaques, primarily on the trunk. Lesions are less than 5 cm in diameter, asymptomatic, and covered with fine, moderately adherent scales. Patient general health remains unaffected.

A distinctive variant known as **digitate dermatosis** features yellowish or fawn-colored, finger-shaped lesions that follow the skin's lines of cleavage, creating a "fingerprint" pattern—often described as resembling a hug that left marks on the trunk. Although individual lesions are narrow, their long axis frequently exceeds 5 cm .

Chronic superficial dermatitis is a synonym for SPP. Digitate lesions with a yellow hue were historically termed *xanthoerythrodermia perstans*.

Box 25-1: Differential Diagnosis of Poikiloderma

- Large-plaque parapsoriasis

- Dermatomyositis
- Lupus erythematosus
- Chronic radiation dermatitis
- Bloom syndrome
- Rothmund-Thomson syndrome
- Dyskeratosis congenita
- Xeroderma pigmentosum

Laboratory Tests

Histopathology

In early large-plaque parapsoriasis (LPP) lesions: - The epidermis is mildly acanthotic and slightly hyperkeratotic with focal parakeratosis. - The dermal lymphocytic infiltrate is perivascular and scattered.

In more advanced lesions: - An interface dermatitis pattern emerges with clear **epidermotropism**. - Lymphocytes may be scattered singly or in clusters, sometimes associated with mild spongiosis.

In poikilodermatous lesions: - Epidermal atrophy is evident. - Dilated blood vessels (telangiectasia) and melanophages are present in the dermis.